Chapter 1: Introduction to Social and Behaviour Change Communication (SBCC)

1.1 The Role of Communication in Social and Behavioural Change

As scholars and practitioners of public health and development, we often grapple with a fundamental question: *Why do people continue to engage in practices detrimental to their health and well-being, even when they know the risks?* The answer lies in the complex web of human psychology, sociology, and environmental factors. Herein lies the critical role of communication.

In the context of social and behavioural change, communication is far more than the mere transmission of information. It is a strategic, participatory catalyst designed to empower individuals and communities to adopt positive behaviours. Communication facilitates dialogue, challenges deeply ingrained social norms, builds self-efficacy, and advocates for structural changes that make healthy behaviours possible and sustainable. It acts as the bridge between scientific evidence and human action, translating abstract health and development goals into actionable, culturally resonant, and socially supported practices.

1.2 The Evolution of the Field: From IEC to BCC to SBCC

The discipline we now call Social and Behaviour Change Communication (SBCC) has undergone a significant paradigm shift over the last five decades. This evolution reflects our deepening understanding of human psychology and social ecology.

Information, Education, and Communication (IEC)

Emerging in the mid-20th century, IEC was built on the "knowledge-deficit model." The foundational assumption was that individuals engage in unhealthy behaviours simply because they lack correct information. IEC campaigns were highly instructional, relying heavily on one-way mass media (e.g., posters, radio broadcasts, and pamphlets) to pour facts into an audience. For example, an IEC campaign might simply state, "Wash your hands to prevent disease." *The Flaw:* IEC suffered from the "KAP-Gap" (Knowledge-Attitude-Practice gap). Research consistently showed that increasing a population's knowledge rarely translates directly into sustained behaviour change.

Behaviour Change Communication (BCC)

Recognizing the limitations of IEC, the field shifted toward BCC in the 1990s. BCC integrated psychological theories—such as the Health Belief Model and the Transtheoretical Model—to target the motivations and attitudes of individuals. It moved beyond just providing facts to addressing the *why* of behaviour. BCC messaging was targeted and aimed at building personal risk perception and self-efficacy. *The Blind Spot:* While highly effective at the individual level, BCC often ignored the socio-economic and cultural environments. Asking a woman to exclusively breastfeed is ineffective if she must return to agricultural work immediately and lacks community support.

Social and Behaviour Change Communication (SBCC)

Today, SBCC represents a holistic, 360-degree approach. It retains the clear messaging of IEC and the motivational psychology of BCC but adds a crucial dimension: **the social ecosystem**. SBCC operates on the Socio-Ecological Model, acknowledging that individual behaviour is influenced by interpersonal networks, community norms, institutional environments, and public policies. SBCC does not just aim to change the individual; it aims to change the social norms and structural barriers surrounding them.

1.3 Key Principles and Characteristics of SBCC

To effectively design and implement SBCC, practitioners must adhere to five core principles. Understanding these principles distinguishes a strategic SBCC campaign from a simple public awareness push.

1. Theory-Driven

SBCC is never based on guesswork or "gut feeling." It relies on established behavioural and social theories to understand the determinants of a specific issue. Theories act as roadmaps, helping practitioners predict how and why people change, which variables to measure, and how to craft the intervention.

- **Further Explanation:** Using a theory helps answer questions like: Is the barrier to change a lack of self-efficacy? Is it peer pressure? Is it a low perception of risk? By identifying the root cause through a theoretical lens (e.g., the Theory of Planned Behaviour, Social Cognitive Theory, or the Health Belief Model), communicators can target the exact psychological or social lever that needs pulling.

- **Example in Action:** If an organization wants to increase condom use among young adults, relying merely on "informational" posters is insufficient. Using the *Theory of Planned Behaviour*, researchers might discover that while knowledge is high, the true barriers are *attitudes* (believing condoms reduce pleasure), *subjective norms* (stigma around buying them), and *perceived behavioural control* (not knowing how to negotiate use with a partner). The SBCC campaign would then specifically target these exact variables—perhaps through role-playing scenarios in peer groups to build negotiation skills.

2. Evidence-Based and Research-Led

Rigorous research is the backbone of SBCC. This principle dictates that every step of a campaign—from conceptualization to evaluation—must be grounded in data.

- **Further Explanation:** This involves intensive *formative research* (using surveys, focus groups, and ethnography) to understand the audience deeply. It requires *audience segmentation*—dividing a broad population into smaller subgroups based on demographics, psychographics, or behaviour to tailor messages precisely. Crucially, it also demands *pre-testing* messages and materials with the target audience before a full launch to ensure cultural resonance and avoid unintended backlash.
- **Example in Action:** Before launching a campaign to improve maternal nutrition in a rural district, formative research might reveal a critical insight: young mothers actually have very little say in what the family eats. The research shows that mothers-in-law control the household food budget and dictate dietary norms. Based on this evidence, the SBCC campaign pivots; rather than solely targeting the pregnant women, the primary audience segmentation shifts to engage and educate the mothers-in-law as the key gatekeepers of nutritional change.

3. The Socio-Ecological Approach

Interventions are designed across multiple levels simultaneously, recognizing that people do not make decisions in a vacuum.

- **Further Explanation:** The Socio-Ecological Model posits that behaviour is influenced at five levels: Individual (knowledge, attitudes), Interpersonal (family, friends), Community (workplaces, neighborhoods, social norms), Institutional/Organizational

(rules, regulations), and Public Policy (laws). Effective SBCC tries to align interventions across as many of these levels as possible.

- **Example in Action:** Consider an SBCC program aimed at reducing tobacco use.
    - *Individual:* Counselling and quit lines to build individual self-efficacy.
    - *Interpersonal:* Support groups involving family members of smokers.
    - *Community:* Working with local leaders to declare community centres smoke-free zones.
    - *Policy:* Advocating for local ordinances that raise taxes on tobacco or restrict advertising near schools.
    - Changing the behaviour is much easier when the entire environment supports the change.

4. Participatory and Community-Led

SBCC treats communities as co-creators of knowledge, not passive recipients. It prioritizes community engagement, dialogue, and capacity building to foster long-term ownership.

- **Further Explanation:** The era of top-down communication ("doing *to* the community") is over; SBCC is about "doing *with* the community." When communities help identify their own problems and design their own solutions, the interventions are more culturally appropriate and sustainable. This involves human-centred design and participatory communication methodologies.
- **Example in Action:** The *Community-Led Total Sanitation (CLTS)* approach is a hallmark of participatory SBCC. Instead of an NGO arriving and building latrines (which often end up being used as storage sheds), facilitators guide community members through an exercise to map out open defecation sites in their own village. Through guided dialogue, the community triggers its own collective sense of disgust and shame, leading them to design, fund, and build their own latrines using local materials. The change is driven by community pride and self-determination, not external directives.

5. Multi-Channel Integration

Effective SBCC uses a strategic mix of mass media, interpersonal communication (IPC), and community mobilization to reinforce messages. This creates a "surround sound" effect.

- **Further Explanation:** People rarely change a deeply ingrained habit after hearing a message just once. They need to encounter consistent messaging from various trusted sources. Mass media raises broad awareness and sets the agenda; community mobilization shifts collective norms; and interpersonal communication provides the tailored, one-on-one support necessary to actually adopt the new behaviour.
- **Example in Action:** A successful national polio vaccination drive relies heavily on multi-channel integration.
    - *Mass Media:* National radio jingles and TV soap opera storylines normalize the vaccine.
    - *Community Mobilization:* Local religious leaders publicly endorse the vaccine during Friday prayers, alleviating community fears and shifting social norms.
    - *Interpersonal Communication:* Female community health workers conduct door-to-door visits to answer specific, private questions from hesitant mothers, providing the final reassurance needed for them to accept the drops for their children.

Case Study in Action: A prime example of these principles in action is documented by Dearden et al. (2023) in their evaluation of a large-scale SBCC intervention in the Lake Zone Region of Tanzania. To address high rates of childhood stunting, the program utilized mass media (radio/TV) combined with robust interpersonal communication through frontline workers. The intervention systematically addressed maternal health, exclusive breastfeeding, and water, sanitation, and hygiene (WASH) practices. The study found that exposure to this multi-channel, ecologically grounded SBCC intervention significantly improved women's knowledge and attitudes regarding infant feeding and male involvement in child-rearing, proving the efficacy of addressing both the individual mother and the broader family dynamic.

1.4 SBCC vs. Traditional Communication Approaches

It is essential to distinguish SBCC from traditional communication approaches (such as corporate public relations or basic health journalism).

Feature	Traditional Communication / IEC	SBCC
Direction of Flow	One-way (Top-down)	Two-way (Dialogic and Participatory)

Primary Goal	Awareness and Information dissemination	Measurable change in behaviours and social norms
Focus	The Message and the Medium	The Audience, their context, and root causes
Locus of Blame	Often inadvertently blames the individual for failing to act	Acknowledges systemic and social barriers to action
Approach	Message-driven	Strategy- and Theory-driven

Case Study in Action: The contrast between traditional medical delivery and an SBCC approach is highlighted in a 2024 study on Mass Drug Administration (MDA) for Neglected Tropical Diseases (NTDs) in Jimma, Ethiopia. Traditionally, MDA campaigns focus purely on the logistical distribution of pills. However, this study evaluated the integration of tailored SBCC into the MDA campaign. By moving beyond traditional "take your medicine" directives to implementing a dialogic SBCC strategy that addressed local perceptions and myths, the intervention achieved large effect sizes in improving both the perception of Onchocerciasis and the uptake of preventive practices among the community (Diriba et al., 2024).

1.5 Ethical Considerations in SBCC

Because SBCC involves an explicit intent to change how people live, think, and interact, it carries profound ethical responsibilities. As practitioners, we must navigate several critical ethical boundaries:

1. **Avoiding Victim-Blaming:** When emphasizing individual behaviour change (e.g., eating healthier), we must ensure we do not stigmatize individuals who face structural barriers (e.g., living in a food desert). SBCC must acknowledge social determinants of health.
2. **Persuasion vs. Coercion:** SBCC aims to persuade and enable, never to manipulate or coerce. Audiences must retain their autonomy to make informed decisions. Using excessive fear appeals or manipulative emotional triggers is ethically questionable and often counterproductive.
3. **Cultural Humility and Imposition:** Who decides what a "positive" behaviour is? SBCC practitioners, often funded by Western or global north institutions, must avoid imposing external values onto local communities. Formative research and community co-design are ethical imperatives to ensure interventions are culturally respectful.
4. **Equity and Access:** Communication campaigns can inadvertently widen health disparities if they only reach those with access to mass media or smartphones. Ethical

SBCC demands a commitment to reaching marginalized and vulnerable populations through accessible channels.

Conclusion

The transition from simply telling people what to do to engaging them in a dialogue about how to change their social realities represents the maturation of our field. As you progress through this text, remember that SBCC is not a rigid formula, but a dynamic, empathetic, and scientifically grounded approach to fostering human dignity and well-being.

1.6 Discussion Questions for Students

1. **The "Surround Sound" Dilemma (Efficacy vs. Autonomy):** Effective SBCC relies heavily on "Multi-Channel Integration" to ensure the target audience encounters the message everywhere—from the radio to their local community leaders, to door-to-door health workers. However, ethical guidelines state we must respect audience autonomy and avoid coercion.

   Does creating an inescapable "surround sound" messaging environment border on psychological coercion? Does true autonomy exist when your entire social ecosystem is systematically engineered to push you toward one specific choice?

2. **Structure vs. Agency (The Limits of the Socio-Ecological Model):** The chapter notes that traditional IEC often "victim-blamed" individuals, whereas SBCC emphasizes the profound impact of structural and social environments on our health choices.

   By leaning so heavily into the Socio-Ecological Model, does SBCC risk stripping individuals of their personal agency and accountability? Where exactly should public health practitioners draw the line between systemic failure and personal responsibility?

3. **The Ethics of Funding and Cultural Imperialism:** Many large-scale SBCC interventions in the Global South are funded and initially conceptualized by institutions based in the Global North. While SBCC practitioners strive for "cultural humility" and participatory design,

   Is it genuinely possible for a foreign-funded public health campaign to be entirely free of cultural imposition? When does promoting a "universally positive health behaviour" cross the line into enforcing Western values on a different culture?

References

Dearden, K., et al. (2023). "The Impact of a Large-Scale Social and Behavior Change Communication Intervention in the Lake Zone Region of Tanzania on Knowledge, Attitudes, and Practices Related to Stunting Prevention." *International Journal of Environmental Research and Public Health*, 20(2), 1214.

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